



Product Requirements Document

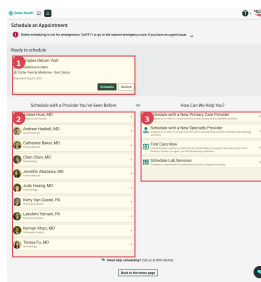
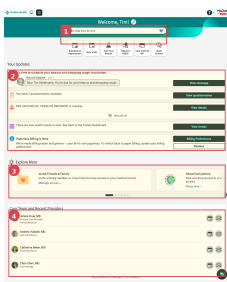
My Health Online Patient Portal Redesign

A patient-centered redesign strategy focused on simplifying navigation, reducing appointment-booking friction, improving task completion, and making high-value health actions easier to complete.

Product area: Sutter Health My Health Online portal

Primary journeys: Home, navigation, appointment scheduling, messages, test results, medication renewals, billing

Design strategy: Shift from feature access to guided action.



1. Executive summary

My Health Online provides broad access to care tasks, health records, messaging, billing, and account services. The core opportunity is not adding more functionality; it is reducing cognitive load and guiding patients to the right next action at the moment they need help.

The current experience behaves like a comprehensive records database. The recommended product direction is to make it behave like a guided health task assistant: clear next steps, patient-language labels, stronger prioritization, and fewer competing pathways.

67%	Current appointment flow completion baseline
5	Core tasks elevated on home and navigation
30-40%	Target reduction in menu-level choices
<3 clicks	Target path to schedule, message, refill, results, or bill

2. Problem statement

Patients often come to the portal when they are ill, anxious, or trying to complete a time-sensitive task. The current system exposes too many options, uses internal healthcare terminology, and makes high-frequency tasks compete with low-frequency settings and administrative content.

- Patients must understand system categories before they can act.
- Appointment scheduling asks patients to choose between provider type, visit type, and care channel before clarifying intent.
- Test results, medications, and billing pages show dense data but provide limited prioritization, plain-language interpretation, or next-step guidance.
- The full navigation is closer to a site map than a decision aid.

3. Product goals and non-goals

Goals	Non-goals
Increase appointment scheduling completion, reduce navigation overload, clarify the patient next best action, improve comprehension of test results and billing status, and make common tasks reachable from the home page.	Do not change clinical policy, medical decisioning, provider availability logic, billing systems of record, or Epic platform constraints in this phase. This PRD defines user experience and content requirements for a redesign concept.

4. Primary users and jobs to be done

User / state	Job to be done	Design implication
Sick or anxious patient	Find the fastest appropriate care option without understanding healthcare system complexity.	Use guided intake and intent-based scheduling choices.
Ongoing care patient	Review results, messages, medications, and upcoming visits quickly.	Create a prioritized Today module with actionable status.

User / state	Job to be done	Design implication
Billing-focused patient	Understand whether money is owed and what account the bill belongs to.	Show a total balance summary first; collapse zero-balance accounts.
Caregiver or family manager	Navigate records, visits, messages, and payment tasks across care contexts.	Keep core tasks persistent; move family sharing into Account unless actively needed.

5. Prioritized requirements

R1 - Simplified global navigation

Reduce the full menu to five patient-centered groups: Care, Messages, Health Record, Billing and Insurance, Account. Add a Popular Actions area for Schedule, Message, Results, Refills, and Bills.

R2 - Home page Today module

Replace the undifferentiated update stream with a prioritized Today section that ranks action-required items above informational updates.

R3 - Intent-based appointment scheduling

Start scheduling with the question: What do you need help with? Route to see my doctor, care today, specialist, lab work, requested follow-up, or I am not sure.

R4 - Patient-language content system

Replace internal labels such as Complex Return Visit, Guarantor, and Request Renewal with plain-language alternatives.

R5 - Results grouping and interpretation

Group test results into New, Needs attention, Labs, Imaging, and Older. Add status labels and provider-reviewed states.

R6 - Medication renewal clarity

Show renewal status badges, simplify top instructions, support bulk renewal selection, and separate remove-from-list from renewal actions.

R7 - Billing summary first

Show total amount due, due date, and payment status before account-level detail. Collapse zero-balance cards by default.

R8 - Message guidance

Replace the generic Send a message entry with message-type choices: medical question, medication question, test result question, billing question, referral question, or technical support.

R9 - Progressive disclosure

Move disclaimers, related links, and educational content into collapsible help sections to keep task pages focused.

R10 - Accessibility and usability polish

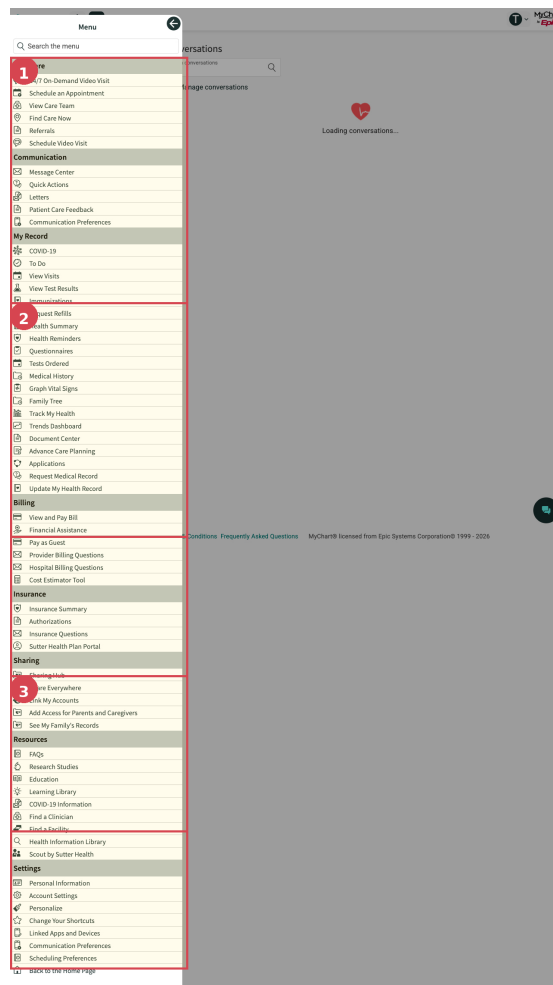
Increase touch target consistency, improve visual hierarchy, reduce repeated row actions, and ensure high contrast for status labels and primary actions.

6. Home page requirements



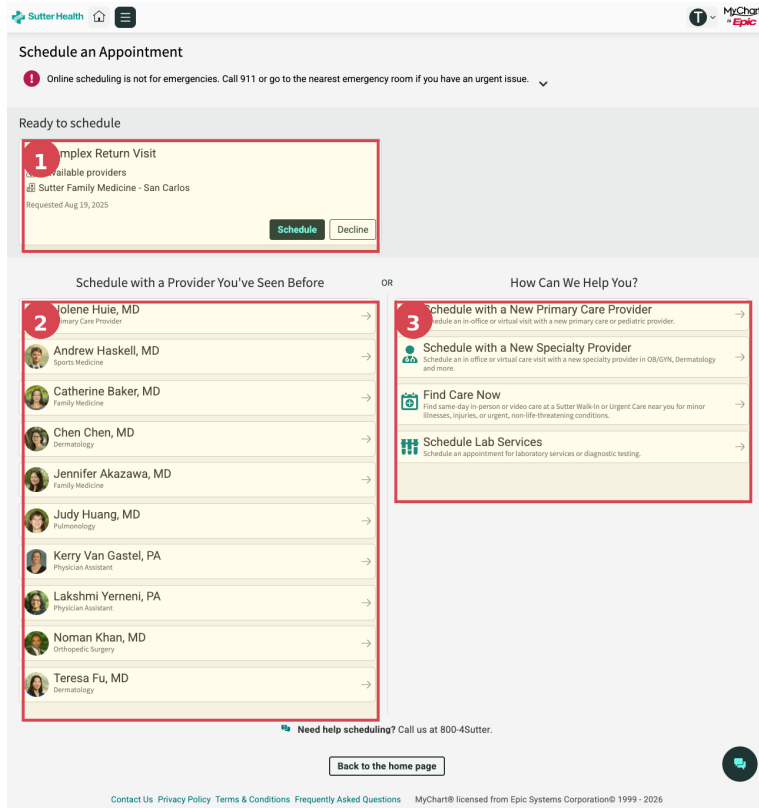
Highlighted issue	Requirement / what needs to be done
1. Search/help field is ambiguous	Rename to Search or ask for help; return direct actions, not only links.
2. Updates are stacked without priority	Create Today module with Needs action, For review, and Information states.
3. Explore More competes with core tasks	Move carousel below the core task area or only show when contextually relevant.
4. Care team list consumes major screen space	Condense care team to primary care provider plus recent providers; keep Schedule and Message actions visible.

7. Global navigation requirements



Highlighted issue	Requirement / what needs to be done
1. Full menu exposes too many destinations	Replace site-map menu with five top-level groups and a Popular Actions section.
2. Health record items are fragmented	Group tests, medications, visits, documents, immunizations, and health summary under Health Record.
3. Resources and settings contain low-frequency items at equal weight	Move low-frequency content behind More and use search for long-tail destinations.

8. Appointment scheduling requirements



Schedule an Appointment

Online scheduling is not for emergencies. Call 911 or go to the nearest emergency room if you have an urgent issue.

Ready to schedule

1. Complex Return Visit
 Available providers
 Sutter Family Medicine - San Carlos
 Requested Aug 15, 2025
[Schedule](#) [Decline](#)

Schedule with a Provider You've Seen Before OR **How Can We Help You?**

2. Schedule with a Provider You've Seen Before

- Jolene Huie, MD (Primary Care Provider)
- Andrew Haskell, MD (Sports Medicine)
- Catherine Baker, MD (Family Medicine)
- Chen Chen, MD (Dermatology)
- Jennifer Akazawa, MD (Family Medicine)
- Judy Huang, MD (Pulmonology)
- Kerry Van Gastel, PA (Physician Assistant)
- Lakshmi Yerneni, PA (Physician Assistant)
- Noman Khan, MD (Orthopedic Surgery)
- Teresa Fu, MD (Dermatology)

3. How Can We Help You?

- Schedule with a New Primary Care Provider**
Schedule an in-office or virtual visit with a new primary care or pediatric provider.
- Schedule with a New Specialty Provider**
Schedule an in-office or virtual care visit with a new specialty provider in OB/GYN, Dermatology and more.
- Find Care Now**
Find same-day in-person or video care at a Sutter Walk-In or Urgent Care near you for minor illnesses, injuries, or urgent, non-life threatening conditions.
- Schedule Lab Services**
Schedule an appointment for laboratory services or diagnostic testing.

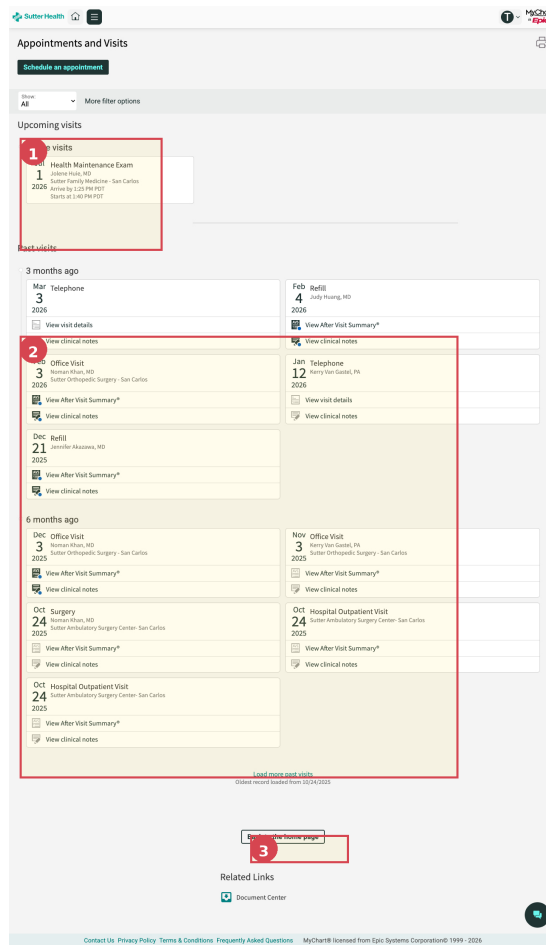
[Need help scheduling? Call us at 800-4Sutter.](#)

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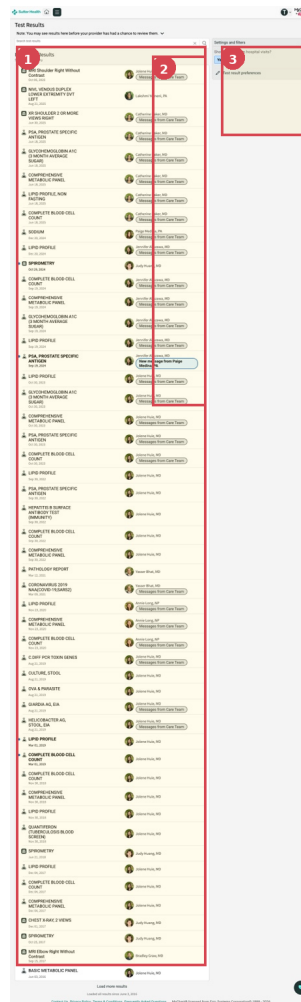
Highlighted issue	Requirement / what needs to be done
1. Complex Return Visit uses internal terminology	Rename to Recommended follow-up visit and explain who requested it and why.
2. Provider-first list assumes the patient knows who to pick	Add intent-first entry points before provider selection.
3. Care-type options compete without guidance	Group care options by need: care today, primary care, specialist, labs, not sure.

9. Appointments and visits requirements



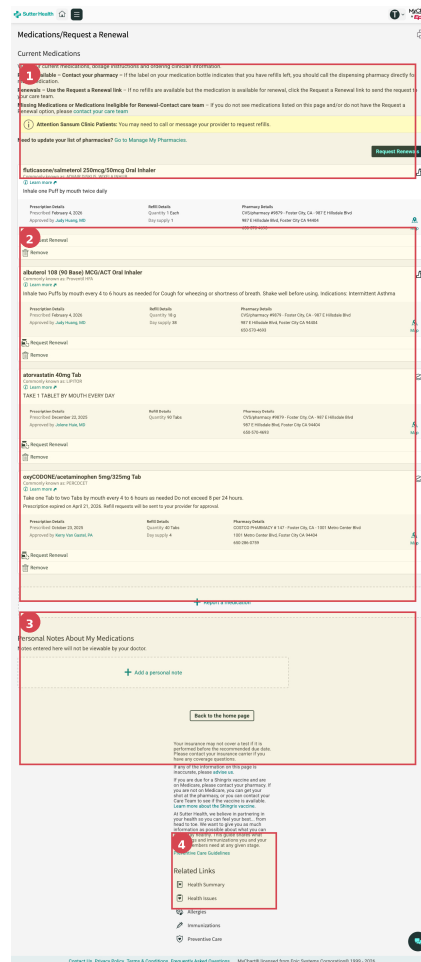
Highlighted issue	Requirement / what needs to be done
1. Upcoming visit is visually underweighted	Make upcoming visit a hero card with check-in, directions, reschedule, cancel, and message actions.
2. Past visits dominate the page	Group past visits by year and add search/filter by provider, visit type, and date.
3. Back to the home page button is legacy navigation	Remove page-level back button and rely on persistent header navigation.

10. Test results requirements



Highlighted issue	Requirement / what needs to be done
1. Long chronological list is hard to scan	Group results and summarize repeated lab panels into latest result plus history/trend.
2. Provider/message controls repeat on every row	Show one contextual Message care team action at result detail or grouped result level.
3. Filters are weak and not task-centered	Add filters for New, Abnormal, Reviewed, Labs, Imaging, Provider, and Date range.

11. Medication renewal requirements

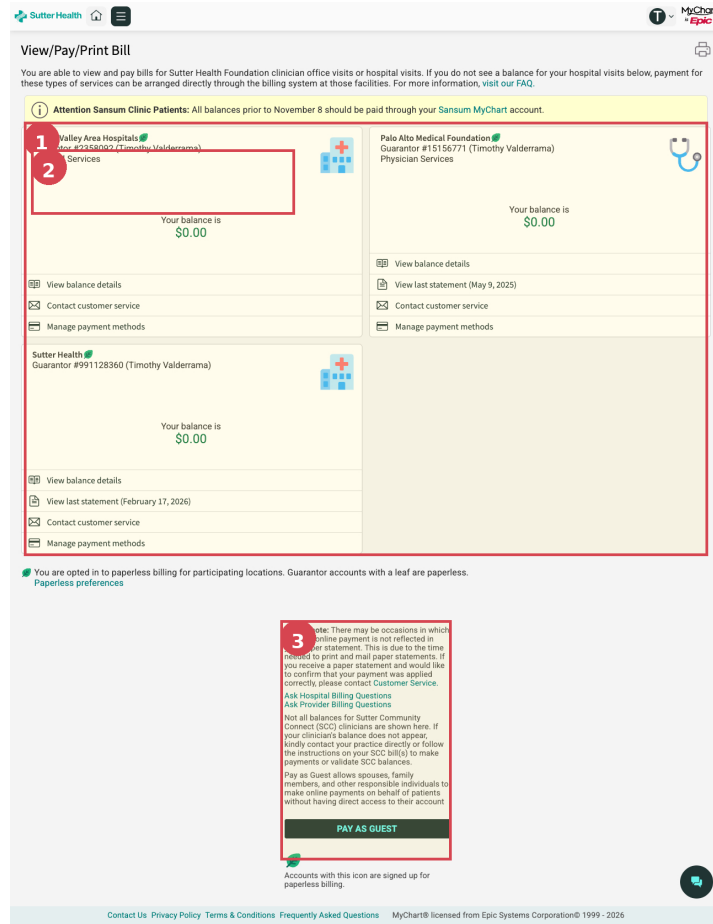


The screenshot shows a web interface for requesting medication renewals. It lists several medications with their details and a 'Request Renewal' button for each. The annotations are as follows:

- 1. Instructional text delays the primary task:** Points to the top section containing instructions on how to use the 'Request a Renewal' link and what to do if no refills are available.
- 2. Medication cards are dense and repetitive:** Points to the medication cards, which contain a lot of repetitive information like 'Pharmacy Details' and 'Refill Details'.
- 3. Report medication and personal notes are visually oversized:** Points to the 'Personal Notes About My Medications' section, which is a large text area.
- 4. Clinical disclaimer area clutters the page:** Points to the bottom section containing a disclaimer and related links.

Highlighted issue	Requirement / what needs to be done
1. Instructional text delays the primary task	Move instructions into How refills work; keep top area focused on renewal actions.
2. Medication cards are dense and repetitive	Add scannable card hierarchy: name, purpose, dose, status, pharmacy, primary action.
3. Report medication and personal notes are visually oversized	Resize secondary actions and place below active medications.
4. Clinical disclaimer area clutters the page	Collapse disclaimers and related links under Need help or Learn more.

12. Billing requirements



View/Pay/Print Bill

You are able to view and pay bills for Sutter Health Foundation clinician office visits or hospital visits. If you do not see a balance for your hospital visits below, payment for these types of services can be arranged directly through the billing system at those facilities. For more information, [visit our FAQ](#).

Attention Sansum Clinic Patients: All balances prior to November 8 should be paid through your **Sansum MyChart** account.

1 Valley Area Hospitals
Guarantor #9358002 (Timothy Valderrama)
Services

Your balance is
\$0.00

View balance details
Contact customer service
Manage payment methods

2 Palo Alto Medical Foundation
Guarantor #15156771 (Timothy Valderrama)
Physician Services

Your balance is
\$0.00

View balance details
View last statement (May 9, 2025)
Contact customer service
Manage payment methods

Sutter Health
Guarantor #991128360 (Timothy Valderrama)

Your balance is
\$0.00

View balance details
View last statement (February 17, 2026)
Contact customer service
Manage payment methods

3 **Pay as Guest**

Note: There may be occasions in which online payment is not reflected in your statement. This is due to the time needed to print and mail paper statements. If you receive a paper statement and would like to confirm that your payment was applied correctly, please contact Customer Service.
Ask Hospital Billing Questions
Ask Provider Billing Questions
Not all balances for Sutter Community Connect (SCC) clinicians are shown here. If your clinician's balance does not appear, kindly contact your practice directly or follow the instructions on your SCC bill(s) to make payments or validate SCC balances.
Pay as Guest allows spouses, family members, and other responsible individuals to make online payments on behalf of patients without having direct access to their account.

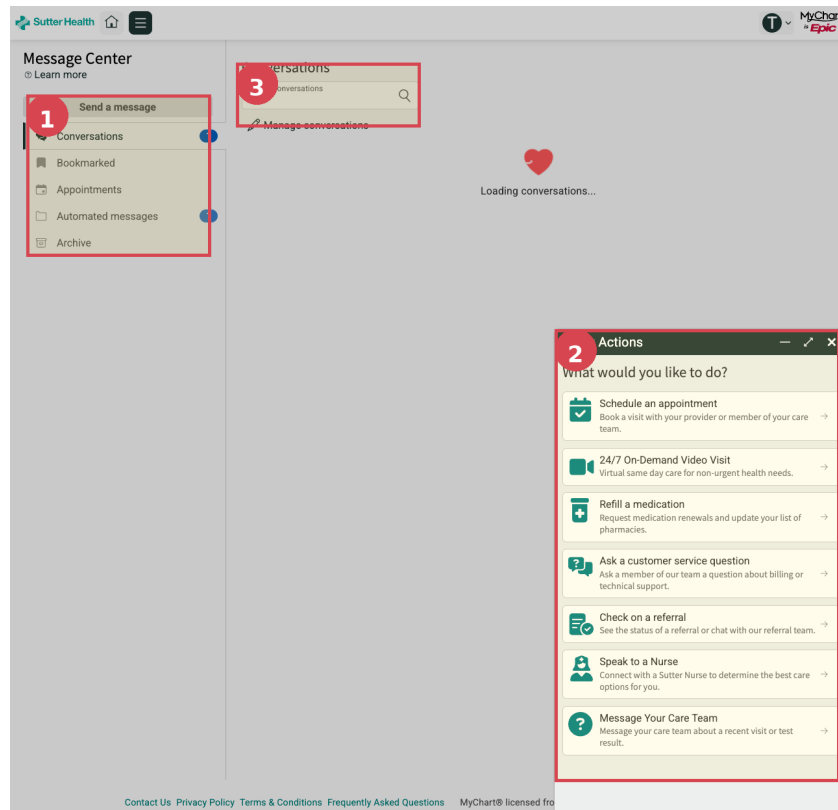
Accounts with this icon are signed up for paperless billing.

You are opted in to paperless billing for participating locations. Guarantor accounts with a leaf are paperless.
[Paperless preferences](#)

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Highlighted issue	Requirement / what needs to be done
1. Multiple zero-balance cards create unnecessary work	Show total amount due first and collapse zero-balance accounts by default.
2. Guarantor is not patient language	Rename to Billing account and explain hospital services versus physician services in plain language.
3. Dense notes and Pay as Guest are disconnected from the signed-in task	Move signed-in payment tasks above disclaimers; place Pay as Guest in a secondary help area.

13. Message Center requirements



Highlighted issue	Requirement / what needs to be done
1. Left navigation is message-system centered	Replace with message intent choices and clearer inbox states.
2. Quick Actions floats over content and duplicates primary tasks	Integrate Quick Actions into page content or home task hub instead of overlay.
3. Conversation search appears before useful empty-state guidance	Use empty state: No conversations yet. Start a message to your care team, billing, referral team, or support.

14. Content model and labeling changes

Current label	Recommended label	Reason
Complex Return Visit	Recommended follow-up visit	Reduces anxiety and explains the task in patient language.
Guarantor	Billing account	Avoids billing-system jargon.
Request a Renewal	Refill or renew medication	Matches what patients commonly say and search for.
View Visits	Appointments and visit history	Combines future and past mental models.
My Record	Health record	Clearer and more explicit.
Provider Billing Questions	Ask about a doctor visit bill	Clarifies difference from hospital bill.
Automated Messages	System messages	Clearer inbox category.

15. Measurement plan

Metric	Target direction	Why it matters
Appointment scheduling completion rate	Increase from 67% baseline	Primary business and patient-access outcome.
Time to schedule appointment	Decrease	Measures friction during the highest-priority journey.
Navigation search usage and zero-result rate	Higher successful search; lower zero-result rate	Shows whether search helps users bypass menu overload.
Message misrouting rate	Decrease	Indicates whether message categories match patient intent.
Refill renewal completion	Increase	Measures clarity of medication task flow.
Billing page support clicks	Decrease when balance is zero or simple	Indicates whether patients understand payment status.
Task success in usability testing	90%+ for top five tasks	Validates redesign before implementation.

16. Implementation phases

Phase 1 - Low-hanging fruit: Navigation restructuring, content label updates, home page Today module, simplified appointment entry labels, collapse secondary content.

Phase 2 - Task-flow redesign: Intent-based scheduling, guided message chooser, test result grouping, medication renewal status, billing summary-first layout.

Phase 3 - Intelligent guidance: Personalized next-best-action logic, global task-aware search, proactive refill and appointment prompts, and user-tested care-assistant interactions.

17. Acceptance criteria

- Top five tasks are accessible from the home page and global navigation without opening the full site-map menu.
- Appointment scheduling starts with patient intent and avoids internal visit-type language on the entry screen.
- Test results are grouped by status and type, with new or abnormal items visually prioritized.
- Medication cards expose renewal eligibility and a clear primary action without requiring users to parse long instructions.
- Billing shows total amount due before account-level detail and hides zero-balance complexity by default.
- All revised labels pass plain-language review and are understandable without healthcare-system knowledge.
- Prototype usability testing demonstrates improved task success, reduced time on task, and lower perceived difficulty.

Closing product principle

My Health Online should move from a comprehensive portal of features to a guided patient operating system: one that tells patients what needs attention, explains what it means, and gives them the fastest safe path to act.